INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Cetuximab + cisplatin + fluorouracil

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
3
Explained in the treatment reference

Regimen source
NCCP 00417
Version 7

> Every 21 days for up to 6 combination cycles; cetuximab is weekly.

Treatment stages

01 **Combination treatment**
Every 3 weeks

02 **Days 8 and 15 within the same combination cycle**
Weekly cetuximab visit

03 **Possible cetuximab-only maintenance, if separately agreed**
Separate weekly plan

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

> Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

Cetuximab + cisplatin + fluorouracil
National Cancer Control Programme (NCCP) 00417

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call oncology immediately for fever, shivering, sudden illness, persistent vomiting, diarrhoea, severe mouth pain or inability to drink. Chest pain during or after fluorouracil, or unusually early severe symptoms, needs urgent assessment even if dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) testing was normal. Report breathing difficulty, swelling, faintness, spreading skin infection or painful eyes promptly. Severe chest pain, severe breathing difficulty, collapse or a seizure is an emergency.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

How your medicines work

Cetuximab with cisplatin and fluorouracil is used to treat squamous head and neck cancer. Discuss the individual aim, likely benefit, symptom control and alternatives with your oncology team.

EGFR-TARGETED ANTIBODY

Cetuximab

Erbitux



Blocks a growth signal on cells

Cetuximab attaches to the epidermal growth factor receptor (EGFR) on cells and blocks growth signals. It is combined with chemotherapy in this plan. Healthy skin and nails also depend on this pathway, explaining some of the characteristic rash, dryness and nail problems. This is a different treatment from checkpoint immunotherapy.

EGFR blocked → Growth messages reduced → Cancer growth restrained

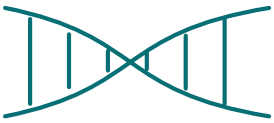
What to watch for

Tell staff immediately about itching, flushing, swelling, wheeze or faintness. Report skin infection, eye pain, new breathlessness or marked cramps.

PLATINUM CHEMOTHERAPY

Cisplatin

Cisplatin



Damages cancer-cell deoxyribonucleic acid (the cell's genetic material) (DNA)

Cisplatin links strands of DNA, the instructions inside cells, making it harder for cancer cells to repair damage and divide. Healthy cells may also be affected. Kidney function, hearing, blood counts and nerve symptoms help the team decide whether the next dose is safe.

DNA damage → Cell repair disrupted → Cancer growth reduced

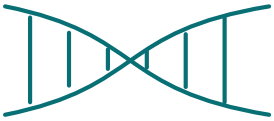
What to watch for

Report reduced urine, persistent vomiting, tinnitus, hearing change or new tingling.

ANTIMETABOLITE CHEMOTHERAPY

Fluorouracil

5-FU



Disrupts production of DNA and RNA

Fluorouracil, often shortened to 5-FU, interferes with the building blocks used to make DNA and RNA, which cells need to grow. It runs through a vein over the prescribed hours. Dihydropyrimidine dehydrogenase (DPD) is an enzyme that breaks it down. Testing for DPD deficiency before treatment reduces risk but cannot rule out every severe reaction.

Cell building blocks disrupted → Cells cannot divide normally → Pump provides prescribed exposure

What to watch for

Report severe diarrhoea, mouth sores, chest pain, sudden confusion, sore palms or soles, or unexpectedly early severe symptoms.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding your diagnosis

Keep the pathology, stage and agreed treatment aim with your records.

Site and type

Ask which head or neck site the cancer started in, what the biopsy shows, how far it has spread and how previous treatment affects the options. HPV/p16 tests apply to some tumour sites; they are different from tests of chemotherapy safety.

Who this plan is for

Treatment of locally advanced or metastatic squamous cell cancer of the head and neck.

Cancer tests and treatment-safety tests

A scan or a tumour-specific test assesses the cancer. Blood counts and kidney or liver tests assess treatment safety. DPD testing concerns fluorouracil metabolism and is not a measure of cancer response.

Understanding the evidence

The EXTREME study tested adding cetuximab to platinum and fluorouracil for recurrent or metastatic squamous head and neck cancer.

The study supports this approach in an appropriate setting. The reported results pooled the platinum options; they are not a separate randomised survival estimate for this particular platinum or for a curative treatment plan. Population: Recurrent or metastatic disease; the study used either cisplatin or carboplatin. Comparison: Platinum and fluorouracil with or without cetuximab. Study populations and individual circumstances differ. The study does not establish that cisplatin and carboplatin give identical outcomes. The current regimen and an individually agreed plan govern treatment duration.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your treatment stages and visit types

Treatment stages

01 **Combination treatment**
Every 3 weeks

02 **Days 8 and 15 within the same combination cycle**
Weekly cetuximab visit

03 **Possible cetuximab-only maintenance, if separately agreed**
Separate weekly plan

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

EVERY 3 WEEKS

Combination treatment

Every 21 days for up to 6 combination cycles; cetuximab is weekly.

Cisplatin is given on Day 1, fluorouracil on Days 1–4, and cetuximab on Days 1, 8 and 15 of each 21-day cycle, for up to 6 combination cycles. The team may then consider weekly cetuximab alone under a separate maintenance plan (NCCP 00207). This is not an automatic continuation.

WEEKLY CETUXIMAB VISIT

Days 8 and 15 within the same combination cycle

Days 8 and 15 of each 21-day combination cycle

These cetuximab-only visits remain part of the same combination cycle. Continue the skin, allergy and symptom checks; use the prescribed visit-specific support plan.

SEPARATE WEEKLY PLAN

Possible cetuximab-only maintenance, if separately agreed

Only after an explicit review and maintenance prescription

The source allows consideration of cetuximab alone after the combination course, under NCCP 00207. Ask for a separate calendar and support/monitoring plan. The six combination stamps do not record maintenance.

Visits within a cycle are not separate courses. The team records delays, omitted doses and any change of plan. A stamp records completed treatment; it does not authorise the next dose.

BASELINE → TREATMENT → FOLLOW-UP

Your treatment, tests and checks

Required regimen tests, symptom-triggered investigations and cancer-response tests serve different purposes. Ask who books each test and who explains the result.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00417 v7: official regimen · EMA: Erbitux product information and patient leaflet

Tests in your plan

Full blood count

WHEN

At baseline and before each cycle

Checks infection-fighting cells, oxygen-carrying cells and platelets.

Kidney and liver tests

WHEN

At baseline and before each cycle

Guide safe treatment and identify organ problems.

DPD assessment

WHEN

Before the first fluorouracil treatment unless already tested

Checks for reduced ability to break down fluorouracil; results affect treatment choice or dose. Testing cannot exclude every severe reaction. Kidney impairment can affect one blood-based test, so the team chooses the appropriate method.

Hearing assessment

If part of your plan

WHEN

At baseline when clinically indicated; review new hearing or balance symptoms during treatment

Investigates hearing loss or tinnitus and helps judge platinum-related risk.

Electrocardiogram (ECG)

If part of your plan

WHEN

Before treatment if heart function is compromised; additional assessment if symptoms warrant

Records the heart's electrical activity. New chest symptoms require assessment even after a reassuring earlier test.

Magnesium and other electrolytes

WHEN

Magnesium at baseline and before each cycle; magnesium, calcium and electrolytes monthly for 2 months after final cetuximab

Detects mineral changes caused by cetuximab; low levels may cause cramps, weakness or heart-rhythm problems.

Imaging and tumour-specific tests

If part of your plan

WHEN

As individually arranged for the disease and symptoms; no universal marker panel or imaging interval

Assesses the cancer rather than whether a treatment-day blood count is safe.

THE WHOLE TREATMENT JOURNEY

Cetuximab + cisplatin + fluorouracil · 21-day cycle: treatment and checks

Locally advanced or metastatic head and neck squamous cell carcinoma.

Cisplatin is given on Day 1, fluorouracil on Days 1–4, and cetuximab on Days 1, 8 and 15 of each 21-day cycle, for up to 6 combination cycles. The team may then consider weekly cetuximab alone under a separate maintenance plan (NCCP 00207). This is not an automatic continuation.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Blood and organ checks NCCP	Full blood count, kidney and liver tests and magnesium; DPD testing before fluorouracil unless already tested. Review previous antibody reactions, red-meat allergy, tick bites and any cetuximab allergy-test results. An ECG is indicated with compromised heart function; hearing tests and creatinine clearance are arranged when indicated.	Full blood count, kidney and liver tests and magnesium before each cycle. Check skin, eyes, bowel symptoms and infusion reactions. Ask about hearing, tinnitus, tingling and hydration at each cycle. Report symptoms before every weekly cetuximab visit, even when chemotherapy is not scheduled.	Report new symptoms promptly; extra tests may be needed.	A held or changed dose needs a new clinical review and relevant repeat results.	After the final cetuximab dose, electrolytes including magnesium and calcium are checked monthly for 2 months. Continue review of persistent skin, kidney, hearing, nerve and nutritional problems. Disease examinations and scans are individually planned.

Care / check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Treatment and appointments NCCP	Confirm the diagnosis, aim, prescription and actual calendar.	Cetuximab starts with a 400 mg/m² loading dose over 120 minutes, then 250 mg/m² over 60 minutes weekly. Chemotherapy starts at least 1 hour after cetuximab finishes. Cisplatin 100 mg/m² is given over 2 hours on Day 1 with prescribed fluids before and after. Fluorouracil is 1,000 mg/m² over 22 hours daily on Days 1–4, or the alternative 2,000 mg/m² over 46–48 hours repeated on Days 3–4. Staff confirm the chosen pump schedule. Plan for monitoring during and for at least 1 hour after each cetuximab infusion under the current medicine information; confirm the observation plan with your unit.	Follow the written home plan and keep pump or line appointments where applicable.	Cisplatin is given on Day 1, fluorouracil on Days 1–4, and cetuximab on Days 1, 8 and 15 of each 21-day cycle, for up to 6 combination cycles. The team may then consider weekly cetuximab alone under a separate maintenance plan (NCCP 00207). This is not an automatic continuation.	Record the last dose and follow-up appointments.
Mouth, weight and swallowing NCCP	Review oral health, weight, nutrition and safe swallowing.	Discuss mouth pain, intake, weight and hydration.	Report poor drinking, choking or weight loss promptly.	Dietitian and swallowing plans change with symptoms and recovery.	Continue dental, nutritional and swallowing support when needed.

Care / check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Cancer assessment NCCP	Agree staging, pathology and any relevant disease tests.	Discuss symptoms and the planned response review.	Scans or examinations may be added if symptoms require them.	The team sets imaging and cancer-assessment timing.	No universal imaging interval or routine tumour-marker panel is imposed by this regimen.

What each check is for

1 Combination treatment

Follow your agreed treatment calendar

Cetuximab with cisplatin and fluorouracil is used to treat squamous head and neck cancer. Discuss the individual aim, likely benefit, symptom control and alternatives with your oncology team.

Ask / act: Confirm active medicines, written support plan and review criteria.

2 Days 8 and 15 within the same combination cycle

Follow your agreed treatment calendar

Cetuximab with cisplatin and fluorouracil is used to treat squamous head and neck cancer. Discuss the individual aim, likely benefit, symptom control and alternatives with your oncology team.

Ask / act: Confirm active medicines, written support plan and review criteria.

3 Possible cetuximab-only maintenance, if separately agreed

Follow your agreed treatment calendar

Cetuximab with cisplatin and fluorouracil is used to treat squamous head and neck cancer. Discuss the individual aim, likely benefit, symptom control and alternatives with your oncology team.

Ask / act: Confirm active medicines, written support plan and review criteria.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your port and treatment pump

From preparing the line to coming home after disconnection. Your team arranges the appointments and shows you what to do.

1 · Before treatment

Confirm access and training

Your team selects the access device and teaches its care, including a port or peripherally inserted central catheter (PICC) if used.

2 · Before Day 1

Write down the infusion plan

Daily 22-hour fluorouracil on Days 1–4, or 2,000 mg/m² over 46–48 hours twice as prescribed; confirm changes and completion.

3 · Treatment days

Staff connect and change infusions

Only trained staff connect, program or change the infusion. Do not calculate or alter the rate yourself.

4 · While connected

Watch the line and device

Call for leakage, pain, swelling, damage, disconnection, unexpected completion or a pump that appears not to run. Follow the unit's device-specific emergency instructions.

5 · At prescribed completion

Attend for disconnection

Completion depends on the actual start time and chosen infusion duration. Do not disconnect or reconnect the line yourself.

6 · Between cycles

Keep the access-care plan

A line may remain between treatments. Keep dressing, flushing and review appointments and report redness, pain, fever or swelling.

Port or PICC: what is the difference?

A port is a small chamber beneath the skin, usually on the chest, attached to a tube ending in a large vein. A special needle goes through the skin into the port when it is used. A PICC enters a vein in the upper arm; a short section remains outside under a dressing. Both can deliver a home infusion. Your team helps choose the device and teaches its care.

Before insertion and during recovery

Tell the team about allergies, blood thinners, infections and any other cancer medicines. Follow the instructions about eating, drinking and transport; do not stop medicines yourself. Port placement uses local anaesthetic, sometimes with sedation or general anaesthetic. The team explains the procedure and risks, including infection, bleeding, clots and insertion complications. Ask how to manage discomfort, protect the wound and resume activity. Report increasing pain, redness, swelling, discharge or fever.

Connection: what the nurse does

The nurse checks the line and treatment, secures the connection and dressing, and starts the pump. Before leaving, you should have the disconnection location and time, contact numbers including out of hours, and the written spill instructions. Ask to be shown what normal pump progress looks like. You do not need to calculate a rate.

Carrying it, sleeping and washing

Use the supplied carrier and the position recommended for your pump. Keep tubing free from pulling and kinks. Ask how to place the pump safely for sleep and how to protect the line and dressing when washing. Do not immerse it or swim. Avoid direct heat and cold. An elastomeric pump has a balloon that shrinks slowly and may have no alarm; an electronic pump has different instructions. Follow the instructions for the device you actually receive.

Disconnection is not port removal

Keep the appointment even if the pump looks empty, unless the team changes the plan. Staff check the pump and line and arrange safe disposal. After flushing and other line care as required, the port needle may be removed and a small dressing applied. The implanted port stays in place until a separate removal procedure is arranged. A PICC remains in place and needs scheduled dressing and flushing care.

Leakage, disconnection or unexpected pump progress

Call the team immediately if the pump leaks, is damaged, finishes unexpectedly early, appears not to run, or the line disconnects. Follow the emergency or spill steps you have been specifically taught, including use of a clamp if instructed. Do not reconnect, change settings or disconnect the pump yourself. Keep other people away from leaked medicine. If medicine touches skin, rinse it off and wash with soap and water, then seek advice.

Pain, swelling or feeling unwell

Contact oncology immediately for pain or swelling around the line, a wet/loose dressing, redness, fever, shivering or suddenly feeling unwell. A painful swollen arm or neck needs urgent advice. Severe chest pain, major breathing difficulty or collapse: call 112 / 999. Do not wait for pump disconnection or the next clinic visit.

The drawings show general steps, not instructions for inserting or accessing a line. Your hospital confirms its own arrangements and the care instructions for your device.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before the first visit

Bring your medicines and allergy list, test dates, treatment calendar and actual emergency contacts. Tell the unit about swallowing, hearing, mobility, transport or communication needs.

STEP 2

Time in the unit

Cetuximab starts with a 400 mg/m² loading dose over 120 minutes, then 250 mg/m² over 60 minutes weekly. Chemotherapy starts at least 1 hour after cetuximab finishes. Cisplatin 100 mg/m² is given over 2 hours on Day 1 with prescribed fluids before and after. Fluorouracil is 1,000 mg/m² over 22 hours daily on Days 1–4, or the alternative 2,000 mg/m² over 46–48 hours repeated on Days 3–4. Staff confirm the chosen pump schedule. Plan for monitoring during and for at least 1 hour after each cetuximab infusion under the current medicine information; confirm the observation plan with your unit.

STEP 4

Before you leave

Check the next treatment and test dates, medicines, pump or line appointments, and who to contact if unwell. Have the team replace and verify every example phone number on the wallet card.

STEP 5

Practical support

Ask about transport, work, costs, caring responsibilities, an interpreter or a support person. Keep changes to the plan in one record.

Recovery, review and longer-term care

Finishing scheduled doses and recovering are different milestones.

Symptoms and follow-up

After the final cetuximab dose, electrolytes including magnesium and calcium are checked monthly for 2 months. Continue review of persistent skin, kidney, hearing, nerve and nutritional problems. Disease examinations and scans are individually planned.

The next cancer assessment

Record who arranges examinations, scans and any tumour-specific tests, and who explains the results. A general reference calendar does not set an appointment or mean a scan is overdue.

Information for other clinicians

Keep the regimen name, last treatment date, current medicines and unresolved side effects available. Tell future clinicians about any earlier head and neck radiotherapy, platinum-related hearing or kidney problems, and significant infusion reactions.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Cetuximab	 **Day 1** Day 1; loading dose at first administration.
Cetuximab	 **Day 8** Weekly cetuximab after review.
Cetuximab	 **Day 15** Weekly cetuximab after review.
Cisplatin and hydration	 **Day 1** 100 mg/m² on Day 1; chemotherapy at least 1 hour after cetuximab.
Fluorouracil infusions	 **Days 1–4** Daily 22-hour or two 46–48-hour options as prescribed.

Treatment sequence: Cisplatin is given on Day 1, fluorouracil on Days 1–4, and cetuximab on Days 1, 8 and 15 of each 21-day cycle, for up to 6 combination cycles. The team may then consider weekly cetuximab alone under a separate maintenance plan (NCCP 00207). This is not an automatic continuation.

Appointments and administration: Cetuximab starts with a 400 mg/m² loading dose over 120 minutes, then 250 mg/m² over 60 minutes weekly. Chemotherapy starts at least 1 hour after cetuximab finishes. Cisplatin 100 mg/m² is given over 2 hours on Day 1 with prescribed fluids before and after. Fluorouracil is 1,000 mg/m² over 22 hours daily on Days 1–4, or the alternative 2,000 mg/m² over 46–48 hours repeated on Days 3–4. Staff confirm the chosen pump schedule. Plan for monitoring during and for at least 1 hour after each cetuximab infusion under the current medicine information; confirm the observation plan with your unit.

Baseline and ongoing checks: Full blood count, kidney and liver tests and magnesium; DPD testing before fluorouracil unless already tested. Review previous antibody reactions, red-meat allergy, tick bites and any cetuximab allergy-test results. An ECG is indicated with compromised heart function; hearing tests and creatinine clearance are arranged when indicated. Full blood count, kidney and liver tests and magnesium before each cycle. Check skin, eyes, bowel symptoms and infusion reactions. Ask about hearing, tinnitus, tingling and hydration at each cycle. Report symptoms before every weekly cetuximab visit, even when chemotherapy is not scheduled.

Recovery and follow-up: After the final cetuximab dose, electrolytes including magnesium and calcium are checked monthly for 2 months. Continue review of persistent skin, kidney, hearing, nerve and nutritional problems. Disease examinations and scans are individually planned.

> **Why these bands stay broad:** This shows the prescribed pattern, not guaranteed symptom dates. Actual dates, delays and pump times must be confirmed by the team. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00417 v7: official regimen ↗

> **There are no guaranteed "safe days".** Your prescription and team's instructions take priority. Contact the team about new symptoms rather than waiting for a scheduled visit.

Between-treatment notes

Day 1–4: Combination and fluorouracil days

Daily 22-hour fluorouracil on Days 1–4, or 2,000 mg/m² over 46–48 hours twice as prescribed; confirm changes and completion. Check the pump appointments and contact the unit for problems.

Day 5–15: Recovery plus weekly cetuximab

Cetuximab is due on Days 8 and 15 after review. Other days are not automatically free of side effects. Report diarrhoea, mouth pain, rash, allergy symptoms or poor drinking before each visit.

Day 16–21: Prepare the next cycle

Continue symptom monitoring and arrange the blood tests required for the next cycle. Ask for revised dates after any hold or delay.

Support around the medicines

1 One written support-medicine plan

Ask for one list separating prevention medicines, rescue treatment and regular medicines. The schedule must match this treatment and visit. Record what you take and whether it helps; do not double a dose, restart an old prescription or add medicines without checking.

2 Preventing sickness

This platinum-containing combination needs an agreed anti-sickness plan. Take prevention medicines at the stated times and keep rescue instructions available. Contact the team for persistent vomiting or inability to drink.

3 Cetuximab premedicines and delayed reactions

An antihistamine and corticosteroid are given before cetuximab to reduce reaction risk. They do not prevent every reaction. Report itching, flushing, swelling, wheeze, dizziness or faintness immediately, including symptoms that start after leaving the unit.

4 Skin, nails, eyes and minerals

Use the skin-care plan from the team, including moisturising and sun protection. Report painful cracks, inflamed nails, pus or spreading redness. Eye pain or visual change needs prompt review. Keep prescribed blood tests for magnesium and other minerals; supplements must follow clinical advice.

5 Hydration around cisplatin

Cisplatin requires prescribed intravenous fluids and mineral replacement when indicated. Follow the team's drinking advice and report much less urine, persistent vomiting or difficulty drinking. Do not alter fluids or water tablets yourself.

6 Food, fluids and weight

Tell the team early about reduced intake or weight loss. A dietitian can adapt food texture, supplements and intake goals. Follow an individual fluid plan, especially with kidney or heart problems.

7 Mouth and swallowing care

Follow the mouth-care and pain-relief plan. Report ulcers, thrush or swallowing difficulty early. Ask for a swallowing assessment if food or drinks make you cough or choke; do not assume a general food texture is safe for you.

8

Fertility, pregnancy and medicine checks

Before treatment, discuss fertility, contraception, pregnancy and breastfeeding, including medicine-specific precautions after treatment. Bring all prescription, over-the-counter and herbal medicines to review. The team must check interactions and allergies.

Treatment choices and your priorities

The consent discussion sets out the personal aim and boundaries.

Expected benefit and alternatives

Cetuximab with cisplatin and fluorouracil is used to treat squamous head and neck cancer. Discuss the individual aim, likely benefit, symptom control and alternatives with your oncology team. Ask about reasonable alternatives, including supportive care, and which outcomes matter most to you. A research result cannot predict an individual outcome.

Changing or stopping treatment

The team may delay, reduce, omit or stop a medicine after symptoms or test results, a change in benefit or your preference. Record which medicine is affected and what is required before restarting. Do not use an old prescription to restart treatment.

Other treatment and off-label use

Surgery or radiotherapy may have a separate role, but they are not automatically added to this infusion calendar. For a locally advanced plan, ask whether any part of treatment is outside its current licensed indication and why it is recommended.

Preparing for your treatment visits

Use the medicine schedule and logistics pages for the exact visit pattern.

Your personal checklist

Check that the treatment name, planned tests, support medicines, transport and urgent contacts are correct. Explain any previous infusion reaction or change in your health before the medicine is prepared.

When dates change

A missed or held dose is not a completed dose. Ask for revised blood-test, treatment and pump dates where relevant. The team must clarify whether an omitted weekly dose changes later dates.

Recording actual treatment

Record the actual date and medicines received, dose changes, test results and symptoms. The wallet is a brief portable record; the passport holds more detail and does not replace hospital records.

Between visits: symptoms and home medicines

Symptoms do not follow a guaranteed calendar.

One usable medicine list

Keep prevention, rescue and regular medicines separately labelled. Note doses actually taken and whether symptoms improve. Check new prescription, over-the-counter and herbal medicines with the team; do not add or repeat old courses without advice.

Nerve symptoms and daily function

Report numbness, tingling, weakness, falls, difficulty doing buttons or changes in walking. Describe what you can no longer do, not just a symptom score. The team may need to change treatment before symptoms worsen.

Mouth, teeth and jaw care

This is the home for detailed mouth and dental advice.

Care during chemotherapy

Use the prescribed gentle oral-care and pain-relief plan. Report ulcers, infection, bleeding or pain that affects eating or drinking. Check with oncology before dental procedures.

If you have had head and neck radiation

If you have had head and neck radiation, tell your dentist before invasive treatment. Continue the advised preventive care because effects on saliva, teeth and healing can persist. If you have not had radiation, follow the plan for your current treatment.

Eating, drinking and maintaining strength

Adapt the plan to what you can manage safely.

Weight and intake

Record changes in weight and what limits eating: soreness, taste change, dry mouth, nausea or fatigue may need different help. A dietitian can adapt meals, supplements or feeding support. Do not wait for substantial weight loss to ask for review.

When fluids or food are not enough

Contact the team for persistent vomiting, dizziness, much less urine, inability to drink or severe diarrhoea. Fluid goals depend on kidney and heart health. If a feeding tube is needed, ask for training, supplies and a clear problem-contact route.

Swallowing, voice and communication

Protect safe swallowing and make communication easier.

When to seek assessment

Report coughing or choking with food or drinks, a wet voice after swallowing, food sticking or repeated chest infections. Ask for speech and language assessment; use only the textures and exercises recommended for you.

Communication support

Tell the team about hearing, speech or voice changes. Request written instructions, an interpreter, communication aids or help involving a support person when useful.

Daily life and personal priorities

Use the support available around you.

Activity, rest and practical needs

Discuss the level of activity you can manage, work, travel and help at home. Ask for assessment if fatigue, pain, weakness or falls disrupt ordinary activities. Bring practical barriers to appointments into the discussion.

Fertility, intimacy and emotional wellbeing

Ask about fertility preservation before treatment and specific pregnancy, contraception and breastfeeding precautions. Tell the team about anxiety, body-image, sexual or relationship concerns; support can be tailored to your priorities.

Smoking and alcohol

Ask for help with smoking cessation and discuss alcohol with the team. Mouth symptoms, medicines and recovery may affect what is appropriate for you.

Your records and additional help

Essential information should remain usable on paper.

Passport and treatment card

Use the passport for the fuller record and the wallet card for the treatment name, contacts and actual completed cycles. Its QR opens this treatment’s information; it does not contain a personal medical record or update paper stamps.

Complete local contacts

Every example telephone number must be replaced and checked by the treating service before patient use. Add daytime, out-of-hours and backup contacts and keep them accessible to a support person.

Reading without a phone

Read the full handbook in its chapter sequence. The treatment, symptom, contact and practical information does not require scanning a QR or following a hyperlink. Add questions to the passport and bring it to review.

Questions worth asking

What is the aim of this treatment for me, and what are the alternatives?

Cetuximab with cisplatin and fluorouracil is used to treat squamous head and neck cancer. Discuss the individual aim, likely benefit, symptom control and alternatives with your oncology team. Ask your team to write down the individual aim and explain reasonable alternatives, their burdens and the uncertainties before you decide.

Which medicines and tests are due at each visit?

Cisplatin is given on Day 1, fluorouracil on Days 1–4, and cetuximab on Days 1, 8 and 15 of each 21-day cycle, for up to 6 combination cycles. The team may then consider weekly cetuximab alone under a separate maintenance plan (NCCP 00207). This is not an automatic continuation. Full blood count, kidney and liver tests and magnesium before each cycle. Check skin, eyes, bowel symptoms and infusion reactions. Ask about hearing, tinnitus, tingling and hydration at each cycle. Report symptoms before every weekly cetuximab visit, even when chemotherapy is not scheduled. Confirm the actual dates with your unit.

Which symptoms mean I should call immediately?

Call oncology immediately for fever, shivering, sudden illness, persistent vomiting, diarrhoea, severe mouth pain or inability to drink. Chest pain during or after fluorouracil, or unusually early severe symptoms, needs urgent assessment even if DPD testing was normal. Report breathing difficulty, swelling, faintness, spreading skin infection or painful eyes promptly. Severe chest pain, severe breathing difficulty, collapse or a seizure is an emergency. Have the team replace the example numbers with verified daytime, out-of-hours and backup contacts before using the card.

How will we decide whether to continue, change or stop?

Your team reviews symptoms, test results, treatment benefit and your priorities. Ask when cancer-response assessment is planned and what would lead to a change. A delay or omitted dose must be recorded and does not count as treatment received.

What support can help me manage treatment?

Ask for a written support-medicine plan and early help with mouth pain, eating, drinking, swallowing, skin or nerve symptoms. Tell the team about travel, work, caring and communication needs. Dietitian, speech and language, nursing and other support can be arranged according to your needs.

Try explaining the plan back: Explain which medicines you receive on each visit. Name the symptoms that mean you should contact oncology immediately. Show the actual contact numbers and say which tests and appointments are next.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · cetuximab / platinum / fluorouracil

Active medicines: Cetuximab + Cisplatin + Fluorouracil

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1; 80 mg orally Days 2 and 3	Once daily on the stated days NCIS reference; review interactions and the actual multi-day infusion plan.
Ondansetron	16 mg orally Day 1	Before chemotherapy Clinician may split to 8 mg twice daily or reduce to 8 mg once daily. Review QT risk and electrolytes.
Dexamethasone · reconcile total steroid prescription	NCIS reference: 12 mg orally Day 1; 8 mg orally Days 2–4	As prescribed Cetuximab also requires a corticosteroid before infusion. One agreed total dose/ route must replace duplicate prescriptions.
Olanzapine	5 mg orally once daily Days 1–4	As prescribed General high-risk NCIS row. Review sedation and metabolic/falls risk; metoclopramide combination increases extrapyramidal risk.
Metoclopramide · duration to be specified	10 mg orally three times daily in the NCIS subsequent-days row	Exact duration and indication must be prescribed The high-risk row does not state an end day or PRN qualifier. Check renal/ neurological suitability; avoid unreviewed stacking with other dopamine antagonists.
Cetuximab antihistamine	Prescriber/pharmacy to specify agent, dose and route	At least one hour before every cetuximab dose The 00417/00418 source specifies the class and timing, not a dose. Confirm the total steroid alongside the antiemetic dexamethasone.
Cisplatin hydration and electrolytes	Individual pre- and post-hydration prescription required	Before and after Day 1 cisplatin Source includes suggested saline/ magnesium guidance; individualise fluids and potassium.

Additional phase-specific support

- Give an antihistamine and a corticosteroid at least one hour before cetuximab; continue this premedication at subsequent doses. Reconcile steroids with any antiemetic prescription to avoid duplication.
- Ask specifically about earlier antibody reactions, red-meat allergy, tick bites and any cetuximab IgE result. Premedication does not eliminate severe or delayed reactions.
- Use a local EGFR-inhibitor skin plan, moisturiser and sun protection; assess painful, infected or intolerable rash and nail changes promptly.
- NCCP allows observation to cease after two reaction-free cetuximab doses, whereas current Erbitux SmPC section 4.2 requires monitoring during and for at least one hour after each infusion. Agree and document the governed observation plan; do not automatically remove observation.
- Prescribe regimen-specific hydration and electrolyte replacement with fluid-balance review. The appropriate volume depends on kidney and cardiac status; a generic printed plan is not an intravenous-fluid prescription.
- Review hearing, tinnitus and peripheral nerve symptoms before each cycle; damage can persist. Review nephrotoxic, ototoxic and other interacting medicines.
- Record the dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) result before the first fluorouracil exposure unless previously tested. Complete deficiency precludes fluorouracil; partial deficiency needs an individual starting dose and careful subsequent review.
- Normal DPD testing cannot exclude all severe fluorouracil toxicity. Explain unexpectedly early or severe diarrhoea, mucositis, neurological symptoms and chest pain.
- Write the actual infusion option, access arrangement, bag or pump changes and disconnect time. Do not equate four treatment days with a single universal pump duration.
- Provide an individually prescribed diarrhoea plan and hand/foot care advice; severe or persistent symptoms require assessment rather than repeated self-treatment.
- Arrange nutrition, weight and swallowing review early. Dietitian and speech and language therapy input should follow the person's needs; swallowing safety cannot be inferred from a general leaflet.
- Use the agreed mouth-care and dental plan. Report painful swallowing, mouth ulceration, weight loss or inability to drink before the next appointment.
- Reconcile prescription, over-the-counter and complementary medicines. Discuss fertility, contraception and breastfeeding using the actual products and individual circumstances.
- Confirm the hospital's urgent contact and fever instructions. Infection, bleeding, severe diarrhoea, dehydration, chest pain or new breathlessness need prompt assessment, not the next routine review.
- Use this high-risk plan for the platinum treatment day. Do not repeat its full antiemetic course automatically at weekly cetuximab-only visits.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Days 8 and 15 · cetuximab-only visit within the combination cycle

Active medicines: Cetuximab

Emetogenic risk: low

Medicine	Dose / route	When
Antihistamine plus corticosteroid	Individual prescribed agent, dose and route	At least one hour before cetuximab Mandatory infusion premedication. Reconcile any low-risk antiemetic need with this steroid; no automatic platinum prophylaxis.
Rescue antiemetic	As individually prescribed	If required Review persistent nausea; do not reuse the Day 1 platinum course.

Additional phase-specific support

- Give an antihistamine and a corticosteroid at least one hour before cetuximab; continue this premedication at subsequent doses. Reconcile steroids with any antiemetic prescription to avoid duplication.
- Ask specifically about earlier antibody reactions, red-meat allergy, tick bites and any cetuximab IgE result. Premedication does not eliminate severe or delayed reactions.
- Use a local EGFR-inhibitor skin plan, moisturiser and sun protection; assess painful, infected or intolerable rash and nail changes promptly.
- NCCP allows observation to cease after two reaction-free cetuximab doses, whereas current Erbitux SmPC section 4.2 requires monitoring during and for at least one hour after each infusion. Agree and document the governed observation plan; do not automatically remove observation.
- Arrange nutrition, weight and swallowing review early. Dietitian and speech and language therapy input should follow the person's needs; swallowing safety cannot be inferred from a general leaflet.
- Use the agreed mouth-care and dental plan. Report painful swallowing, mouth ulceration, weight loss or inability to drink before the next appointment.
- Reconcile prescription, over-the-counter and complementary medicines. Discuss fertility, contraception and breastfeeding using the actual products and individual circumstances.
- Confirm the hospital's urgent contact and fever instructions. Infection, bleeding, severe diarrhoea, dehydration, chest pain or new breathlessness need prompt assessment, not the next routine review.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Separately confirmed cetuximab maintenance · regimen 00207

Active medicines: Cetuximab

Emetogenic risk: low

Medicine	Dose / route	When
Antihistamine plus corticosteroid	Use the separately authorised 00207 prescription	Before cetuximab as prescribed Continuation is optional and requires a new agreed plan; do not carry cisplatin hydration or fluorouracil pump prescriptions forward.

Additional phase-specific support

- Give an antihistamine and a corticosteroid at least one hour before cetuximab; continue this premedication at subsequent doses. Reconcile steroids with any antiemetic prescription to avoid duplication.

- Ask specifically about earlier antibody reactions, red-meat allergy, tick bites and any cetuximab IgE result. Premedication does not eliminate severe or delayed reactions.
- Use a local EGFR-inhibitor skin plan, moisturiser and sun protection; assess painful, infected or intolerable rash and nail changes promptly.
- NCCP allows observation to cease after two reaction-free cetuximab doses, whereas current Erbitux SmPC section 4.2 requires monitoring during and for at least one hour after each infusion. Agree and document the governed observation plan; do not automatically remove observation.
- Arrange nutrition, weight and swallowing review early. Dietitian and speech and language therapy input should follow the person's needs; swallowing safety cannot be inferred from a general leaflet.
- Use the agreed mouth-care and dental plan. Report painful swallowing, mouth ulceration, weight loss or inability to drink before the next appointment.
- Reconcile prescription, over-the-counter and complementary medicines. Discuss fertility, contraception and breastfeeding using the actual products and individual circumstances.
- Confirm the hospital's urgent contact and fever instructions. Infection, bleeding, severe diarrhoea, dehydration, chest pain or new breathlessness need prompt assessment, not the next routine review.
- NCCP 00417 allows consideration of maintenance under 00207. Confirm the current monotherapy prescription, dose history and observation requirements.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

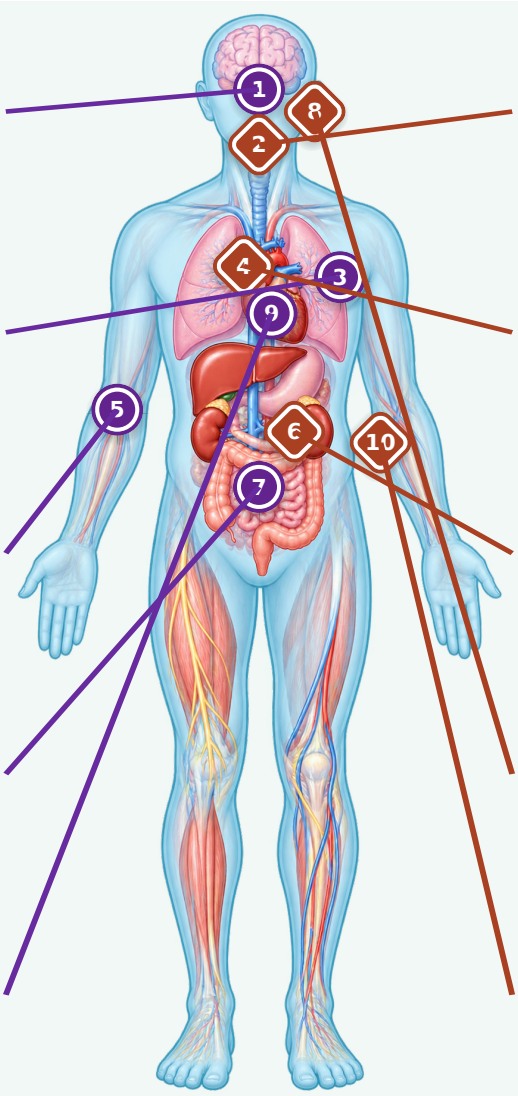
1 Eyes
Cetuximab
Report painful or red eyes, light sensitivity or a change in vision promptly.

3 Lungs / breathing
Cetuximab
Report new cough or breathlessness. Wheeze, swelling or faintness may be an infusion reaction.

5 Skin / nails / hands / feet
Cetuximab + fluorouracil
Report rash, cracked skin, painful nails or sore palms/soles. Spreading redness, pus or fever needs early review.

7 Bowels / hydration
Fluorouracil
Call early for diarrhoea, severe tummy pain, dizziness or inability to keep fluids down.

9 Blood cells / infection
Chemotherapy
Fever, shivering, sudden illness, unusual bruising or bleeding needs prompt oncology advice.



2 Mouth / swallowing
Fluorouracil
Report ulcers, painful swallowing or difficulty drinking; early severe symptoms need urgent review.

4 Heart / chest
Fluorouracil
Chest pain, tightness, palpitations or collapse needs urgent assessment during or after an infusion.

6 Minerals / fluids
Cisplatin + cetuximab
Kidney and mineral tests matter. Report reduced urine, cramps, marked weakness or palpitations.

8 Hearing / balance
Cisplatin
Report tinnitus, hearing loss or balance change before another dose; some changes may persist.

10 Infusion / access site
Cetuximab + chemotherapy
Tell staff about itching, swelling, wheeze or faintness immediately. Delayed reactions can occur after leaving.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call oncology immediately for fever, shivering, sudden illness, persistent vomiting, diarrhoea, severe mouth pain or inability to drink. Chest pain during or after fluorouracil, or unusually early severe symptoms, needs urgent assessment even if DPD testing was normal. Report breathing difficulty, swelling, faintness, spreading skin infection or painful eyes promptly. Severe chest pain, severe breathing difficulty, collapse or a seizure is an emergency.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00417 v7 · pNCCP 00417: treatment and supportive-care sections; NCIS v8 pp4–5 where used**
   https://healthservice.hse.ie/documents/6975/417_Cetux_CIS_5-FU_v7.pdf
2. **EMA: Erbitux product information and patient leaflet**
   https://www.ema.europa.eu/en/documents/product-information/erbitux-epar-product-information_en.pdf
3. **eviQ: port**
   https://www.eviq.org.au/p/507
4. **eviQ: peripherally inserted central catheter (PICC)**
   https://www.eviq.org.au/p/543
5. **eviQ: pump**
   https://www.eviq.org.au/p/502
6. **NCI: head and neck cancers**
   https://www.cancer.gov/types/head-and-neck/head-neck-fact-sheet
7. **NCI: oral complications of cancer therapies, patient version**
   https://www.cancer.gov/about-cancer/treatment/side-effects/mouth-throat/oral-complications-pdq
8. **NCI: nutrition during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
9. **EXTREME: platinum chemotherapy with or without cetuximab**
   https://pubmed.ncbi.nlm.nih.gov/18784101/
10. **NCCP 00417 and current NCIS/local supportive-care guidance**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf